coursera

Depressive and Bipolar Disorders

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders
- Focus of the Course — Reading • 2 min
- Classifying Psychopathology — Reading • 4 min
- Culture and Psychopathology — Reading • 4 min
- Culture and Intersectionality — Reading • 4 min
- Culture and Stigma — Reading • 4 min
- Check Your Understanding: Culture and Mood Disorders — Practice Assignment • Grade: 100%

Lesson 2: Introduction to Depressive and Bipolar Disorders
- Depressive and Bipolar Disorders — Video • 6 min
- Living with a Mood Disorder: A Case Study — Reading • 10 min
- Overview of Depressive and Bipolar Disorders — Reading • 10 min
- Check Your Understanding: Introduction to Depressive and Bipolar Disorders — Practice Assignment • Grade: 80%

Key Takeaways
- Key Takeaways: Opening to Depressive and Bipolar Disorders — Reading • 10 min

Module 2 Quiz
- Module Quiz: Opening to Depressive and Bipolar Disorders — Graded Assignment • Grade: 87.5%

Module 3
Depressive and Bipolar Disorders

Lesson 3: Overview of Depressive Disorders
- Depression: A Case Study — Reading • 5 min
- What Does Depression Feel Like? — Reading • 7 min
- Diagnostic Criteria and Prevalence for Depressive Disorders — Reading • 5 min
- Check Your Understanding: Overview of Depressive Disorders — Practice Assignment • 12 min

Lesson 4: Overview of Bipolar Disorders
- Bipolar: A Case Study — Reading • 5 min
- What Does Bipolar Feel Like? — Reading • 7 min
- **Diagnostic Criteria and Prevalence for Bipolar Disorders** — Reading • 5 min
- Check Your Understanding: Overview of Bipolar Disorders — Practice Assignment • 13 min

Key Takeaways
- Key Takeaways: Depressive and Bipolar Disorders — Reading • 10 min

Module 3 Quiz
- Module Quiz: Depressive and Bipolar Disorders — Graded Assignment • 16 min

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

Module 6
Treatments for Depressive and Bipolar Disorders

Module 7
Suicide

Module 8
Conclusion

Module 9
Cumulative Quiz

Module 10
Course Resources

Module 11
PsycLearn Essentials APA Student Resources

Diagnostic Criteria and Prevalence for Bipolar Disorders

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Diagnostic Criteria and Prevalence

Bipolar disorders are a group of disorders where symptoms of depression and mania or hypomania alternate. *ICD-11* and *DSM-5* list several types of bipolar disorders, which vary in terms of length and severity of symptoms. The most commonly diagnosed forms are bipolar I disorder, bipolar II disorder, and cyclothymic disorder. They can be distinguished based on whether episodes are severe, moderate, or mild, the duration of the episodes, and whether episodes include psychotic features or not. Psychotic features entail a loss of contact with reality, which can include delusions, false beliefs, irrational belief systems, or hallucinations. A bipolar diagnosis can also be accompanied by a specifier for rapid cycling, if there are four or more cycles within a 12-month period, or a specifier for mixed episodes, if cycling between episodes occurs within days, on the same day, or even simultaneously.

Bipolar I Disorder

Bipolar I disorder is defined by the experience of at least one manic or mixed episode lasting at least one week. Some episodes may be hypomanic or depressive. The typical course of the disorder is characterized by recurrent depressive and manic or mixed episodes. Hypomanic episodes are not necessary for a diagnosis of bipolar I disorder. Approximately three-quarters of patients with bipolar I disorder experience psychotic symptoms at some point in their lives (van Bergen et al., 2019). Symptoms may also include:

- inflated self-esteem or grandiosity,
- decreased need for sleep,
- more talkative than usual or pressure to keep talking,
- flight of ideas or subjective experience that thoughts are racing,
- distractibility,
- increase in goal-directed activity,
- impulsive and reckless behavior.

Merikangas et al. (2011) found a prevalence rate of about 0.6% for bipolar I from a survey of 11 countries, which included Brazil, Colombia, Mexico, the United States, Bulgaria, Romania, China, India, Japan, Lebanon, and New Zealand. In an earlier meta-analysis, a prevalence rate of 1.06% was found (Clemente et al., 2015) for 15 studies that included 6 studies from the United States, 3 from China, 2 from Canada, 1 from New Zealand, 1 from Chile, 1 from Australia, 1 from Italy, and 1 from Singapore. A study conducted in countries in Africa yielded somewhat similar, although more variable, prevalence rates of between 0.1% and 1.83% (Esan & Esan, 2016). Various factors, such as ethnicity, culture, and differences in diagnostic criteria used in the studies may contribute to the noted variability (Rowland & Marwaha, 2018). Gender differences in bipolar disorder are minimal (See DiFlorio & Jones, 2010); however, depression is often the predominant episode experienced by women whereas the more prominent episode is mania for men (Lopez-Zurbano & Gonzalez-Pinto, 2019). In addition, substance abuse tends to be a common comorbid disorder for men but eating disorders tend to be more common for women (Suominen et al., 2009). Experiencing episodes of mixed features is common among bipolar patients. In one study, two-thirds of individuals with bipolar disorder with depressive episodes had concurrent manic symptoms (Goldberg et al., 2009). Research has discovered that bipolar patients with mixed features develop the disorder earlier in life, are more likely to be diagnosed with bipolar I disorder, and have a higher number of suicide attempts than bipolar patients without mixed symptoms (Goldberg et al., 2009; Solé et al., 2017).

Bipolar II Disorder

Bipolar II disorder is diagnosed by recurring mood fluctuations with at least one severe depressive event and at least one episode of hypomania but without a history of manic or mixed episodes. Notably, episodes of hypomania do not impair one's social life or job. As a result, when individuals with bipolar II disorder seek treatment, they typically do so because of the depressive component of the disorder.

Bipolar II disorder is distinct from bipolar I disorder in two other ways. First, in addition to the hypomanic episode, at least one severe depressive episode is necessary for a diagnosis of bipolar II disorder. Second, unlike the manic episodes seen in bipolar I disorder, the hypomanic episodes in bipolar II disorder lack the psychotic features sometimes experienced in the manic phases of bipolar I disorder.

For many years, bipolar II disorder was thought to be a milder version of bipolar I disorder (Judd et al., 2003). More recent research suggests that this is inaccurate (Simonsen et al., 2008). First, research suggests that many of the deficits stemming from bipolar II disorder are caused by depression, not hypomania, and research has demonstrated that patients with bipolar II disorder experience more depressive episodes than those with bipolar I disorder (Baek et al., 2011; Judd et al., 2003; Karanti et al., 2020). In addition, differences in symptoms of bipolar I disorder tend to be minimal between men and women, but depressive episodes in bipolar II disorder are more common in women (Bauer & Pfennig, 2005; Benazzi, 1999). Research on the prevalence rates for bipolar II disorder indicates that it is 0.4% of the global population (Rowland & Marwaha, 2018), which is slightly lower than the prevalence for bipolar I disorder (Merikangas et al., 2011).

Cyclothymic Disorder

In between the normal mood fluctuations that people experience every day and the manic, hypomanic, and severe depressive states common in bipolar disorder are states that don't quite qualify as hypomanic or depressive, but that are beyond people's typical experiences. For people whose moods exist in this middle ground, another diagnosis can be used: **cyclothymic disorder** (or, informally **cyclothymia**). In this disorder, people alternate between highs that are not quite hypomanic and lows that are not severe depressive episodes. However, these mood swings are strong enough to impair the person's ability to function in their life and cause distress—defining characteristics of a psychological disorder.

To receive a diagnosis of cyclothymic disorder, the symptoms must have persisted for at least two years. In addition, there must be no history of manic or mixed episodes and the severity and duration of the hypomanic symptoms may not meet the full definitional requirements of a hypomanic episode. Finally, the depressive symptoms are typically milder and more short-lived than those that meet the diagnostic requirements for a depressive episode.

Calling cyclothymia the mildest of the bipolar disorders minimizes the impairments that people with cyclothymia experience. They can still have all the symptoms common in bipolar patients such as impulse control problems, pressured speech, and hypersexuality when in an elevated state and intense sadness, anhedonia, and suicidal ideation during the depressive state (Perugi et al., 2015).

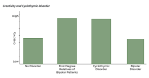
Lifetime prevalence for cyclothymic disorder is estimated to be 0.4%-1% globally and occurs equally often in men and women (Bielecki, 2023). However, the authors note that prevalence rates may actually be higher. Research has suggested that symptoms that cause distress but do not meet the thresholds for diagnosis of bipolar I disorder or bipolar II disorder are far more pervasive (Van Meter et al., 2012), supporting speculation that cyclothymic disorder is the most prevalent bipolar disorder (Van Meter et al., 2017). The picture this creates is of individuals who fluctuate between subthreshold symptoms for hypomania and depression and, therefore, who are less likely to be diagnosed with a disorder and less likely to seek or receive treatment. Though the symptoms of cyclothymic disorder are milder than with bipolar I or bipolar II disorders, they nonetheless cause distress and impair daily functioning (Van Meter et al., 2012).

Cyclothymic Disorder and Creativity

It's long been remarked that artistic genius and psychological disorders go hand in hand. In fact, enhanced creativity seems to be correlated with cyclothymic disorder. Both historically and more recently, researchers have sought to understand how the two traits are related.

Most frequently, researchers studied this link by giving creativity tests to four types of participants: those with bipolar disorder, those with cyclothymia, those with no psychological disorder, and those who are first degree relatives (parents, children, or siblings) of people with bipolar disorder. The image, *Creativity and Cyclothymic Disorder*, illustrates the overall difference in creative output between these different groups. The consistent finding is that people with cyclothymia and their first-degree relatives of people with bipolar disorder show higher degrees of creativity than people with bipolar disorder and people with no psychological disorder (Johnson et al., 2012; Richards et al., 1988). From this, researchers have concluded that mood flexibility, and perhaps the energy that comes with elevated states, is an aid in creativity (Greenwood, 2020).

Creativity and Cyclothymic Disorder



Note. Individuals diagnosed with cyclothymic disorder and individuals who are first-degree relatives of bipolar patients score higher in assessments of creativity than patients diagnosed with bipolar disorder and individuals with no clinical diagnosis.

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